



DEPARTMENT OF REGULATORY AGENCIES

Division of Insurance

LIFE, ACCIDENT AND HEALTH, Series 4-10

3 CCR 702-4 Series 4-10

[Editor's Notes follow the text of the rules at the end of this CCR Document.]

Regulation 4-10-01 HEALTH CARE SHARING PLAN OR ARRANGEMENT REPORTING REQUIREMENTS

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Section 1 Authority

This regulation is being promulgated and adopted by the Commissioner of Insurance under the authority of §§ 10-1-108(7) 10-1-109, 10-16-109, and 10-16-107.4(4), C.R.S.

Section 2 Scope and Purpose

The purpose of this regulation is to establish the data reporting requirements applicable to all health care sharing plans or arrangements (HCSPAs) offering or that intend to offer products to facilitate payment or reimbursement of health-care costs or services for residents of this state.

Section 3 Applicability

This regulation shall apply to all HCSPAs offering or marketing products to facilitate payment or reimbursement of health-care costs or services for residents of this state.

Section 4 Definitions

- A. "Administrative expenses" shall mean costs incurred to operate and support the functioning of the health care sharing plan or arrangement. This includes but is not limited to bank fees, staff salaries, data processing, sales, management of health care expense submissions, marketing, outreach, and enrollment efforts. This includes fees, commissions, and remuneration paid to contractors or third parties that acted on behalf of the HCSPA to facilitate administrative operations.
- B. "CORA" shall mean the Colorado Open Records Act (§ 24-72-201, et seq, C.R.S.).
- C. "Filing date" shall mean, for the purposes of this regulation, the day after the Health Care Sharing Report "HCSR" is received at the Division of Insurance (Division).

- D. "Health care costs" or "health care expenses" shall mean any amount billed by a health care provider for health care services or related products or any amount billed by a pharmacy.
- E. "Health care provider" shall have the same meaning as found at § 10-16-102(56), C.R.S. "Health care provider" includes telehealth and direct primary care providers for the purposes of this regulation.
- F. "Health care services" means any services included in or incidental to the furnishing of medical, behavioral, mental health, or substance use disorder care; dental care; optometric care; hospitalization; or nursing home care to an individual, as well as the furnishing to any person of any other services for the purpose of preventing, alleviating, curing, or healing human physical illness or injury, or behavioral, mental health, or substance use disorders. "Health-care services" includes the rendering of the services through the use of telehealth, as defined in § 10-16-123 (4)(e), C.R.S.
- G. "Health care sharing plan" or "health care sharing arrangement" or "plan" or "arrangement" or "HCSPA" shall mean any organization that offers or markets products to facilitate payment or reimbursement of health-care costs or services for one (1) or more residents of Colorado. This does not include direct primary care agreements as defined in § 6-23-101, C.R.S.; consumer payment plans offered directly between a provider and patient (or patient's responsible party); businesses used to facilitate the plan's operations such as reimbursement handling, cost containment vendors, data processing; and crowdfunded sources that do not require ongoing membership fees, share requirements, or dues for the purposes of payment for and/or reimbursement of health care services.
- H. "Health Care Sharing Report" or "HCSR" shall mean the report required to be filed with the Commissioner pursuant to § 10-16-107.4(1), C.R.S.
- I. "Health Care Sharing Plan or Arrangement Reporting template" or "template" shall mean the data reporting template created and distributed by the Division for the purposes of collecting data per § 10-16-107.4, C.R.S.
- J. "Insurance producer" or "producer" shall have the same meaning as found at § 10-2-103(6), C.R.S., with the exception that for the purposes of this regulation it does not include § 10-2-103(6)(b), C.R.S.
- K. "Product(s)" shall mean, for the purposes of this regulation, the services covered as a package under a membership plan, tier, or level.
- L. "Program expenses" shall mean any service by the HCSPA or its contractors that, while not direct medical care, contributes to the care and overall experiences of HCSPA's participants. This includes but is not limited to coaching and wellness programs, care navigation, care coordination, medical review, quality improvement efforts, cost containment, reimbursement handling, and bill negotiations. This includes fees, commissions, and remuneration paid to contractors that acted on behalf of the HCSPA to facilitate program expenses.
- M. "Third party" shall mean contractors that are associated with or assist the plan or arrangement in offering or enrolling Colorado residents as participants in the plan or arrangement.

Section 5 Rules

- A. All health care sharing plans or arrangements shall:
1. Complete the required HCSR by emailing the Division:

- a. A completed copy of the Health Care Sharing Plan or Arrangement Reporting template, which is provided on the Division's website and contains the data reporting required by § 10-16-107.4(1)(a), C.R.S., including program expenses;
 - b. The Health Care Sharing Plan or Arrangement Reporting Attestation in Appendix A;
 - c. Copies of consumer facing and marketing materials used in Colorado to promote the plan or arrangement associated with the calendar year the plan or arrangement is reporting on. This includes but is not limited to a link to the HCSPA's website(s) used for marketing, plan or arrangement benefit or product descriptions, plan or arrangement member guidelines, application form, and other materials that explain the plan or arrangement;
 - d. An organizational chart of the HCSPA with a list of officers, directors, and their roles;
 - e. Copies of all training materials provided to a producer; and
 - f. Copies of all training materials provided to a third party.
 2. Complete the required HCSR by March 1 of each year with data for the prior calendar year.
 3. Provide accurate contact information for the individual responsible for completing the data reporting form.
- B. Confidentiality**
1. All data filings submitted shall be considered public and shall be open to public inspection, unless the information may be considered confidential pursuant to § 24-72-204, C.R.S.
 2. The entire filing cannot be held as confidential.
 3. A "Confidentiality Index" shall be completed if the health care sharing plan or arrangement desires confidential treatment of any information submitted. Information identified as confidential shall be marked as such and shall be submitted separately from non-confidential material. If a CORA request is received requesting the information identified as confidential, the Division will notify the health care sharing plan or arrangement prior to sharing any information that the plan or arrangement may have identified as confidential in the confidentiality index.
- C.** Where the Division finds the health care sharing plan or arrangement's HCSR to be deficient or incomplete, the Division shall notify the contact person identified in § 10-16-107.4(1)(a)(XVIII), C.R.S. The Division shall allow the HCSPA thirty (30) days from the date of notice to remedy the deficiency. Failure to remedy the deficiency may result in the levy of a fine not to exceed \$5,000 per day. If the HCSPA does not remedy the deficiency or deficiencies within thirty days after the initial fine is levied, the commissioner may issue a cease-and-desist order in accordance with section § 10-3-904.5 C.R.S.

Section 6 Severability

If any provision of this regulation or the application of it to any person or circumstance is for any reason held to be invalid, the remainder of this regulation shall not be affected.

Section 7 Enforcement

Noncompliance with this regulation may result in the imposition of any of the sanctions made available in the Colorado statutes pertaining to the business of insurance, or other laws, which include the imposition of civil penalties, issuance of cease-and-desist orders, subject to the requirements of due process.

Section 8 Effective Date

This regulation shall become effective on April 30, 2024.

Section 9 History

New regulation effective April 30, 2024.

Appendix A Health Care Sharing Plan or Arrangement Reporting Attestation

Health Care Sharing Plan or Arrangement Reporting Attestation

Timeline to respond to incomplete submissions

If the Organization subject to the requirements of § 10-16-107.4, C.R.S fails to submit the information or certification required by § 10-16-107.4, C.R.S, the submission is incomplete. The Commissioner of Insurance shall make a determination of completeness no later than forty-five (45) days after the filing date of the submission. If the commissioner has not informed the Organization of any deficiencies in the submission within forty-five (45) days after receiving the submission, the submission is considered complete.

If the Commissioner determines that a person fails to comply with the requirements of § 10-16-107.4, C.R.S, the Commissioner shall: (a) notify the Organization that the submission is incomplete and enumerate in the notification each deficiency found in the person's submission; and (b) allow the Organization thirty (30) days after notice of the incomplete submission to remedy the deficiency found in the submission. If the Organization does not remedy the deficiency within the thirty-day period, the Commissioner may levy a fine not to exceed five thousand dollars (\$5,000) per day. If the Organization does not remedy the deficiency or deficiencies within thirty (30) days after the initial fine is levied, the Commissioner may issue a cease-and-desist order in accordance with section § 10-3-904.5, C.R.S.

☐ By clicking here, you attest that you read the above statement and are aware of the timeline to respond to the Commissioner and the Division if this submission is deemed incomplete.

☐ By clicking here, you attest that all data required to be submitted, per § 10-16-107.4, C.R.S, have been submitted to the Division. Including submitting of: (1) supplemental, training, and marketing materials; (2) organizational chart of the HCSA with a list of officers, directors, and their roles; (3) copies of any training materials provided to a producer; (4) training materials provided to a third party; and (5) the Reporting Template.

☐ By clicking here, you, an officer of the Organization, attest that the information provided in this reporting template, to the best of your good-faith knowledge and belief, is accurate and satisfies the requirements of § 10-16-107.4, C.R.S.

Digital Signature

Date

Print name and role within organization

Email address

Editor's Notes

3 CCR 702-4 has been divided into smaller sections for ease of use. Versions prior to 09/01/2011 and rule history are located in the first section, 3 CCR 702-4. Prior versions can be accessed from the All Versions list on the rule's current version page. To view versions effective after 09/01/2011, select the desired part of the rule, for example 3 CCR 702-4 Series 4-1, or 3 CCR 702-4 Series 4-6.

History

[For history of this section, see Editor's Notes in the first section, 3 CCR 702-4]